NCCP 00232 · clinician draft

Vinorelbine IV · non-small-cell lung cancer

> **Treatment scope · NCCP 00232** Lung monotherapy context only. The breast guide and its evidence remain separate. Discuss the exact disease setting and alternatives with the treating team.
>
> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Vinorelbine IV · non-small-cell lung cancer · support plan

Active medicines: Vinorelbine

Emetogenic risk: Minimal IV risk

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Individual antiemetic plan differs by route; list routine and rescue medicines separately.
- Discuss constipation prevention, hydration/diet where appropriate and prescribed laxatives; regular opioid use increases the need for a bowel plan.
- No routine steroid premedication, G-CSF, antibacterial, antiviral or PJP prophylaxis is mandated by these NCCP monotherapy documents. Consider additional prevention based on individual risk/local policy; do not create a generic automatic prescription.
- Gastric protection is a consideration for high gastrointestinal-bleed risk, not a universal drug.
- Confirm strong CYP3A4 inhibitors/inducers, P-gp modulators, phenytoin, itraconazole, warfarin and live vaccines. Yellow fever vaccine is contraindicated. Avoid grapefruit per NCCP; do not self-start supplements.
- No routine prophylaxis required by NCCP minimal-risk pathway.
- Prescriber selects an as-needed medicine according to symptoms, constipation, QT/interactions and local policy. Reassess before future doses if nausea/vomiting occurs.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00232 v5 · p5 · NCCP antiemetic guidance v6 · minimal IV pathway

Sources checked 2026-11-02 · SACTCheck v0.84.29 · Regimen-level drafts; prescriber and oncology-pharmacy verification required